

Health Care Utilization and Immigration Enforcement

Midterm Report

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1 Abstract

Background:

Objective: To quantify the impact of immigration enforcement intensity on health care utilization among immigrant populations, specifically evaluating whether the removal of “sensitive location” protections and increased ICE activity function as structural barriers to

medical and preventive care.

Methods:

Results:

Conclusions:

2 Introduction

Immigration-related fears appear to be negatively affecting health care access in the United States (Kaiser Family Foundation 2025). Recent survey evidence indicates that nearly half (48%) of likely undocumented immigrant adults and 14% of immigrant adults overall report that they or a family member have avoided seeking medical care due to immigration-related concerns. These findings suggest that immigration enforcement may function as a structural barrier to health care access, even for individuals who are not directly targeted by enforcement actions. In January 2025, federal policy changes reversed prior protections that designated hospitals and other health care facilities as “sensitive locations,” limiting enforcement activity in these settings (Department of Homeland Security 2025). The removal of these protections may plausibly increase fears associated with seeking care, particularly in communities with visible or intensified U.S. Immigration and Customs Enforcement (ICE) activity.

Motivated by these developments, this project investigates the relationship between immigration enforcement intensity and health care utilization. Specifically, we seek to answer the question: *Does increased ICE enforcement correlate with reduced health care utilization?* We hypothesize that higher levels of local enforcement activity are associated with greater health care avoidance, as individuals may be less willing to leave their homes or engage with formal institutions perceived as potential sites of immigration enforcement.

2.1 Literature Review

Research has documented significant negative effects of immigration enforcement on health care utilization among immigrant populations. At the state level, Friedman and Venkataramani (2021) demonstrated that Hispanic patients were less likely to report having had a regular provider or annual checkup following increased ICE activity in their state. Notably, these behavioral changes were not observed among non-Hispanic adults, a group substantially less vulnerable to deportation enforcement. This differential impact underscores how enforcement intensity functions as a racialized barrier to care. At a more granular level, (Eastus et al. 2025) assessed the relationship between core-based statistical areas (composed of entire

counties or county equivalents (U.S. Census Bureau 2026)) level immigration enforcement and self-rated health among Latinos and found that Latino adults living in areas with the most requests had 24% higher odds of fair/poor health relative to those in the lowest quartile, suggesting that enforcement may have measurable effects on health outcomes through its impact on health care access and utilization.

Provider-level evidence corroborates these patterns. An online survey found that 48% of primary care and emergency medicine providers observed negative effects of ICE enforcement on the health and health access of immigrant patients (Hacker et al. 2012), suggesting that enforcement creates measurable disruptions to clinical practice and patient engagement. Another survey of clinicians found that post-2016 anti-immigrant policies and rhetoric negatively affected health care-seeking behaviors among immigrant patients, where half reported persistent fear among their immigrant patients, an effect that was observed at a higher rate in border states than non-border states (Saadi et al. 2021).

Qualitative and community-based research has illuminated the mechanisms through which enforcement affects health care access. A 2009 study in Everett, Massachusetts found that enhanced immigration enforcement impacted immigrant health both emotionally and structurally, with deportation fears manifesting as concerns about cooperation between local law enforcement and ICE, as well as anxiety about lacking required documentation for insurance and health care enrollment (Hacker et al. 2011). Similarly, research examining an immigration raid in a Midwestern Latino community found that the enforcement action was associated with higher levels of immigration enforcement stress and lower self-rated health among affected residents (Lopez et al. 2017).

While prior studies have documented enforcement effects through local case studies or state-level analyses, research examining the relationship on a national scale at granular geographic and temporal levels remains limited. This project seeks to fill this gap by leveraging national datasets on ICE detention activity and health care utilization to analyze the relationship between enforcement intensity and health care access across geographies and over time.

3 Methods

See Appendix A for detailed Python code used in data processing and analysis.

3.1 Data

We have identified several datasets that may be useful for our analysis:

1. Detention Facilities Average Daily Population (Transactional Records Access Clearinghouse 2026)

This data is available from the Transactional Records Access Clearinghouse (TRAC) and provides information on the average daily population of ICE detention facilities, broken down by city, state, ZIP code, and date from September 2019 through present. This dataset will allow us to measure the intensity of local immigration enforcement activity over time and across geographies.

2. Immigration and Customs Enforcement Detention (Transactional Records Access Clearinghouse 2019)

This data is also available from TRAC and provides monthly counts of individuals detained by ICE at the county level from March 2015 through July 2019. This data also provides counts of individuals detained by ICE by various subgroups, including citizenship status, facility type, when they entered the facility, conviction level, gender, and more. This dataset will complement the first by providing additional granularity on enforcement activity and allowing us to explore potential differential impacts across subpopulations.

3. Routine checkup within the past year among adults (Centers for Disease Control and Prevention 2025)

This data is available from CDC's PLACES data and provides information on the percentage of adults who reported having a routine checkup within the past year, broken down by county, zip, census tract, and year from 2020-2025. This dataset will serve as our primary outcome variable, allowing us to assess health care utilization patterns in relation to local enforcement intensity.

3.2 Analysis

We plan to use Python/PySpark to build panel data across datasets, geographies, and years. We will then use modeling techniques to analyze the relationship between immigration enforcement intensity and health care utilization. We will control for potential confounding factors such as socioeconomic status, demographic characteristics, and local health care infrastructure. We will also explore potential heterogeneity in the relationship by subpopulations, such as by citizenship status or facility type. A potential visualization of our analysis could be a series of maps showing the geographic distribution of enforcement intensity and health care utilization, as well as scatter plots or regression lines illustrating the relationship between these variables.

4 Results

- Results will identify specific geographic areas in need of health outreach.
- Findings highlight the hidden public health costs of immigration enforcement.
- The study provides evidence for maintaining healthcare facilities as safe zones.
- Research helps clinicians understand why immigrant patients may miss preventive care.

5 Discussion

6 Conclusion

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